Audio file

Topic_ Exam 2 Kahoot 2026-10-18 13_48.mp4

Transcript

00:00:02

Your exam to Kahoot that's gonna be voiced over so that you guys can review it, starting with some math questions.

00:00:12

So I'm trying to see if I can.

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Shuffle these, but I can't.

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So let's go in order.

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So we're going 62 kilograms and we're wanting to know how many pounds do they weigh rounding to the nearest tenth.

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So remember, if we're going from kilograms to pounds, our pounds need to be bigger than our kilograms.

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So we would do 62 * 2.2 and we get 136.4.

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OK, now we have a.

00:00:42

actual medication, we have an order to give 600 milligrams.

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What we have available is 300 milligrams per one milliliter.

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And we need to figure out how many mils is the nurse going to give.

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So with this nurse is going to give, if you're a formula method, you're going to do desired.

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We desire to give 600 divided by what do I have on hand?

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300.

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And you get that you're going to give two mils for this.

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We're just going to put the number.

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Next math question is a fluids question.

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So we have 1500 mils of normal saline and we're going to give that over six hours.

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And so we're looking at how many mils per hour will the nurse program into the pump.

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So we know we're working on the pump, so we want to be in our hours and we have hours in our question.

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So we're just going to take our 1500 divided by 6 and we get 250 mils per hour.

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Next one, I forgot to change it to be a type so you have some options.

00:01:43

We need to give 25 milligrams of a medication.

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The available concentration is 7.5 milligrams per milliliters.

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And we want to know how many milligrams will the nurse administer.

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We're just going to take 25 divided by my 7.5.

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We're multiplying it by 1 mil because my volume is 1 and we get 3.3.

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I think this might be the last math question.

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This is our drips.

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So we're supposed to give 300 mils of azithromycin over 90 minutes.

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And my tubing has a drop factor of 10 drops per mil.

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So we want to know how many drops per minute.

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If you're looking at these options, you can automatically get rid of the blue because remember, we can't do a fraction of a drop.

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So then we just take our 300 divided by minutes because remember, we want drops per minute.

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or GTT per minute, so we have to make sure this is in a minute.

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If this said an hour and a half, we would need to convert to 90 minutes.

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So 300 divided by 90 times my drip factor of 10, and we get 33.33333, but we can't do a fraction of a drop, so we're going to pick 33.

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Okay, now we have a select all that apply.

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Which of the following are alternatives to restraints?

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we can try to keep the room door open.

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We can conceal the IV site.

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We can decrease stimulation, and we can encourage family involvement.

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These are all great things that we can do before we use restraints.

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Remember, patients have the right to be in the least restrictive environment, the least restrained.

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So if we can try these things first, we're going to try these things.

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We're going to document what the outcome is, if they work, and if they're not working and the patient's still harmful for themselves or others,

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or they're pulling out a line that they need, pulling out their oxygen, pulling their feeding tube, something that's gonna cause harm to themselves and their care.

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Then after we try some of these alternatives and we've documented that we've tried them, we would move on to doing a restraint.

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A nurse encounters a fire on a unit using the RACE acronym, which steps should we take?

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So with our race, that's going to be rescue if we're able, alarm, contain, and extinguish.

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We want to make sure if we have the ability to help somebody get them out of the fire first, then we're going to pull that alarm, contain the fire if we can by closing doors, and then extinguish the fire with our fire extinguisher.

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Remember our fire extinguisher, the acronym is pass.

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You pull the pin, you're going to aim at the base, squeeze the handle, and sweep.

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Which of the following oxygen delivery devices would you first put on your client with COPD that has an SPO2 of 88?

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So we have our red option is a nasal cannula.

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This looks like a simple mask.

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This looks like it could be a non rebreather, maybe a partial rebreather, hard to tell if there's a valve.

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And then this is the green one is an airplane mask.

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So we know we're not going to put our airplane mask.

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Our risk with the blue and the yellow is first of all, we don't know what this patient's baseline is with COPD, but if we put them on this simple mask or this partial or non-rebreather, they're going to breathe in their own CO2.

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And remember, patients with COPD already have high CO2.

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I'll let the name help you.

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COPD CO2.

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It's too high already.

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I don't want them to breathe in any more of that CO2.

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So we would put them on a nasal cannula.

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If their baseline is normally higher than this and they're having trouble breathing and I put them on this nasal cannula and it does not work, then we'd probably go to BiPAP.

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We're never going to put them on a non-rebreather.

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We don't want them to breathe in that own CO2.

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Encouraging tripoding position and per slip breathing are appropriate nursing interventions for COPD exacerbation.

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This is true.

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We want to do these things.

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Tripodding is going to help them feel like they're opening up those lungs, getting nice big breaths in, and that purse slip breathing is helping them breathe out more CO2.

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So remember, purse slip breathing, breathe in through their nose, purse their lips, and blow out.

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That's going to get rid of more CO2 and syndespirometer is the opposite.

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We take a slow, deep breath in, try to open everything up and get more oxygen in our lungs.

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The nurse should encourage a client with COPD to turn their oxygen up to at least 8 liters nasal cannula when they're short of breath.

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This is false we don't want our patients with COPD to turn their oxygen up over 6 liters.

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If we put it up more than six liters, it can knock out their drive to breathe and then they're going to be in more trouble than they were before.

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So nasal cannula for our COPD patients only up to six.

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We're not putting them on anything higher.

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If they're having trouble and they're already on the six liters, then we're probably going to look for another alternative breathing solution.

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Which pain scale would we use for a patient who is alert and oriented times 4 and able to verbally communicate?

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So faces, that's our Wong-Bakers.

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That's going to be more for our little kids.

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It could be for patients maybe if they can't tell us a number or maybe if they don't understand the number scale if they're not developmentally appropriate for an adult.

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But if we have a patient that's alert and oriented times 4, they're an adult, they're developmentally appropriate.

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We can use the 0 to 10.

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Black is going to be the faces, legs, arms, cry, consolability.

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We use this more for our babies.

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And then we're obviously not just going to take our best guess.

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So we have select all that apply.

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What do you need to do when a patient has a PCA?

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And so we're selecting everything.

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So verify initiating and change it with a second RN.

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Yes.

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We want to make sure that we are setting up that pump correct.

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If we make any changes, we're making sure we have another nurse look over it with us before we document, before we connect it.

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I mean, before we hit go, we're going to make sure we go ahead and verify with that second nurse.

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We still need to monitor vital signs because they are getting a narcotic.

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So we're still going to keep an eye on their vitals, make sure we don't over sedate them, educate the family that they can press the button if the patient's too sleepy.

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Nope, only the patient can press the button.

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Family members cannot press it, especially if their patients too sleepy.

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They may not need more pain medicine or maybe they're already over sedated.

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So we don't want family making that judgment call.

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Educate patient on how to use the button for sure.

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We definitely want to do that.

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OK, chronic pain is protective in nature.

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True or false?

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So this is false.

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Chronic pain doesn't serve a purpose.

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It doesn't tell my body, hey, you hurt yourself here.

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Stop.

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Be careful.

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Sometimes it serves no purpose.

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We don't know why it's hurting.

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We can't find the reason.

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Acute pain is the one that's protective in nature because it's telling us, hey, you hurt your ankle.

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You tripped and you sprained it limp because it hurts.

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Don't put weight on this yet.

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Or if you break your arm, your body's not going to let you pick up a case of water.

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It's going to not let you use that arm.

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So acute pain is protecting us.

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It's letting us know, hey, something is damaged or something is diseased.

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Something's going on with this part of my body.

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For aphasia, which classification of aphasia is defined by the client having an inability to name common objects.

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So if the patient can't name a common object, they have trouble expressing themselves.

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Let the names of these aphasias help you.

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Expressive, the patient can't express what they want to say.

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We might ask them, we might hold up a pen and ask them what this is called and they can't get the word out.

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So that would be they're having trouble expressing themselves.

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Receptive is our other big one.

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This is where the patient can't receive what we're saying.

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What they say might be fluent and it might sound like a sentence, but it either doesn't answer the question, it doesn't make sense in the context, it could have made-up words.

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Global is a combination where they have trouble expressing what they want to say and an inability to get words out, but also an inability to understand what we're telling them.

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And then emotional is just a distractor I made-up.

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Which of the following is an example of a sensory deficit?

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So a client becoming overwhelmed in a noisy environment, that would be a sensory overload.

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Isolation, client in isolation feels lonely and bored.

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That would be a deprivation.

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They're not getting enough of stimulation.

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High pitched hearing, difficulty hearing, reporting difficulty hearing high pitched tones.

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That would be a deficit.

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There's a problem with the organ, the ear.

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It's not working the way it should be and therefore it's causing an issue in how we get that sensation in.

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And then anxiety in overcrowded room, that's gonna be our overload.

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So this one is our blue option.

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So remember, sensory deficit would be things like hard of hearing, blindness, having issues where they can't feel things because they have neuropathy in their fingers.

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So there's an actual issue with the organ, not the environment.

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True or false?

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Integrative therapy involves using just the complementary medicine such as herbal supplements to treat a medical condition.

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This is false because integrative therapy, we're integrating more than one type of treatment.

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So we're using this complementary medicine with our traditional kind of Western medicine.

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So maybe for this patient that's treating a medical condition with herbal supplements, maybe they're also taking

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ibuprofen because they have arthritis.

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They're taking fish oil and ibuprofen.

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Or another big one is with sunburns, patients will take, will use aloe lotion and they'll take ibuprofen for discomfort.

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Smoking patients, they might do a nicotine patch and hypnosis.

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People with back pain might take medicine prescribed by a doctor and see a chiropractor.

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So it's combining these two different methods into one.

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How should we communicate with someone who is hard of hearing?

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We want to talk loud in a low tone.

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So we talk low and loud.

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We don't want to talk slowly because sometimes if they're reading our lips, if we slow our speech down too much, it can distort that.

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We don't want to talk quickly because they may not hear it.

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And we don't want to talk quietly and quickly.

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They're not going to be able to hear it.

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So low and loud.

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What patient would we not provide chiropractic care?

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So which one would we not recommend they go?

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Patient with a headache, they can see a chiropractor.

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Acute pain, chiropractor might be fine.

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GI upset, fine.

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But a patient with a bone fracture, we don't want them going and seeing a chiropractor.

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So osteoporosis is a big one.

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We don't want them seeing chiropractors.

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If someone has a recent spine surgery, we don't want them seeing a chiropractor or any kind of spinal issue.

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Which event would be classified as a sentinel or a never event?

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So wrong dose given, client had no harm.

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That's not a sentinel event.

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That's a patient safety event.

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We don't want this to happen, but we know sometimes it does.

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But it's not a never event because there was no harm.

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Wrong patient med caught before it was given.

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That's a near miss.

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So we don't have to think of that as a never event.

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A client slid to the floor, no injury.

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There was no injury, so we're okay.

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It's still a patient safety event.

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We're still gonna report it.

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We're still gonna track it.

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We're gonna talk about what we could do, fill out that incident report, but it's not considered a never event because the patient didn't get hurt.

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Now if that was said client slid to the floor and broke their leg or had a scratch or did something, then we could talk.

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Our answer here is the stage 3 pressure injury.

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The part that we carry about is this acquired after admission.

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Stage 3 and 4 pressure ulcers should not be acquired after they're admitted.

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That is a never event.

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A client can't afford her medications and has unstable housing.

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So what referral should we put in?

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We should refer them to the social worker.

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So we would want to contact social work, put in a referral, talk to them about the situation because they might be able to help find some resources for either housing, for medication, for both.

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That social worker is going to be a great help.

00:15:01

Which action is appropriate for a client at high risk for falls?

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We can't raise all four side rails because that's considered a restraint.

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The Morse fall scale on admission only, that's incorrect.

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We should be doing that twice a shift, every, not twice a shift, once a shift, twice a day if our nurses are working 12 hour shifts.

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So every shift, a nurse that has a shift with that patient, they should go ahead and complete it.

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We put yellow wristbands on for fall risks, but we want to leave the bed low and locked and make sure that call light is within reach and then also make sure we educate the client how to use the call light.

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A tornado warning is issued.

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A client can't get out of bed.

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So what are we going to do?

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Well, we're not going to open the blinds.

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Let them watch.

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Watch the weather.

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That's a big St.

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Louis thing or Midwest thing where those tornado and people go stand on their porch.

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But we're not going to leave the blinds open.

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We're going to close the blinds.

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We don't want to take the client to the basement by the elevator.

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That's going to be the worst case scenario.

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Move the bed next to the door with blankets off.

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I like to move the bed but not the blankets off.

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We want to move the bed away from the window and cover them with blankets because if the window were to break and there's glass flying, we want to protect our client.

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We have an active shooter on the unit and there's a clear exit path.

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The nurse should.

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We should be running out, leaving belongings and not wait for others.

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Remember, you're hiding only if you can't leave safely.

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You're not going to wait for anybody or gather anybody.

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If you see people, you can have them follow you, but you're not going to gather them.

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And we're not going to call 911 from the nurse's station first.

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Your job is to get out if you have a clear path.

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A client on seizure precautions begins to seize.

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What's our first action?

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We don't want to put in a padded tongue blade because that can actually be a risk and the patient can hurt themselves on that.

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We don't want to hold their arms down.

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We don't want to like restrain them.

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We just want to keep them safe.

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We're definitely not leaving the room to get oxygen.

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If a patient is on seizure precautions, they should already have oxygen ready at the bedside.

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as well as suction, padded side rails, everything that we might need an ambu bag just in case.

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We want to turn the client onto their side because if they were to vomit or have anything come up, we don't want them to aspirate on that.

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Which must the nurse document for a client in restraints?

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This is going to be this first option, alternatives that were tried and failed.

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We're not documenting the family's opinion, names of visitors, or the client's diet order.

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That's already going to be an order in.

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We're not documenting it.

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We're documenting.

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What did we try that didn't work that made this patient need a restraint?

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Maybe they have some kind of chest tube or whatever kind of tube this is in this picture for this patient and they keep pulling at it.

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We need to keep them safe.

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So we're going to document that we tried distraction and we tried reorientation, we tried education and we tried a sitter and they're still pulling at it.

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So we're going to document alternatives.

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We're going to document the behaviors that led to this restraint.

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We wanna document a good assessment before we put the restraints on.

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We wanna document once we put the restraints on, what was the patient's behavior?

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How did they act afterwards?

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Did they calm down and stop pulling at the tube or did they keep trying to pull at the tube?

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We have a post-op ICU patient that's agitated, anxious, they're unable to rest.

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So what intervention would be best for them?

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Our key in this is that they're unable to rest.

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Anxious and agitation, anxious can kind of go with both our deprivation and our overload, but we're looking at this agitated, unable to rest part.

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So this person seems to me like they are in overload.

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They have too much stimulation.

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So we're not going to turn on the TV for more or encourage more visitors.

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And we don't want to keep lights on all night because we want to maintain that day night cycle.

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But we're going to cluster care.

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If we need to go in that room at 8 and at 9 and at 10, well, let's look and see.

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Can we go in at 9 and do everything all at once and then get out of the room?

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Which situation would be considered a deprivation rather than a deficit?

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So remember, deprivation is we're missing that stimulation.

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Not that we can't receive it, but that stimulation is not enough.

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It's not sufficient or gone.

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So our correct option is this green one, the quiet ICU room with no windows.

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The patient can still take in all the senses, but it's an issue with the actual environment.

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So remember, deprivation is environment.

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deficit would be there's an actual issue with the patient's sensory organs.

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What teaching is the priority for a client with diabetic neuropathy?

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So we talked about diabetic neuropathy is where a lot of times patients when their blood sugars are out of control, they get damaged to the small vessels and the nerves.

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usually starts in their fingers and their toes and it can work its way up.

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So we think about patients, we're kind of combining our hygiene, what we talked about with diabetic foot care and our peripheral neuropathy concepts here.

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With the peripheral neuropathy, the biggest thing is that they may not feel their feet.

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So we don't want them to soak their feet in hot water daily.

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We don't want them to walk barefoot at home because they might not feel it.

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We don't want to use a heating pad on their feet because they may not feel it.

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But if they're going to take a bath, we want to check the water with the temperature that's keeping them safe.

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A client says pain, leg, bad, and they're frustrated.

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So we know they have some kind of aphasia.

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Which action would be the most appropriate?

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We don't want to finish the sentence for the client.

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We want to let them try.

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We don't think that we're going to

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figure out exactly what they want to say.

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We're going to encourage them to keep in, keep trying.

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We're not going to be like, oh, your leg is hurting pretty badly.

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We're not going to finish it for them.

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We don't want to treat them as cognitively impaired because of patients with these kind of aphasia.

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This is a patient would have expressive aphasia.

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They're having trouble expressing themselves.

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We don't want to think they're cognitively impaired because they might not be.

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They just can't get the words out.

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Speaking slowly and loudly, they don't have any trouble hearing.

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They just can't get the words.

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So we want to use yes and no questions and a picture board.

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So maybe they say pain, leg, bad.

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And we can say your leg is hurting.

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You can tell me yes or no.

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Or we can show a picture board and they can point to pain and leg and maybe point to a number.

00:22:04

How should the nurse describe a meal tray to a client with low vision?

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This is our first option.

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Use clock positions for each food.

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So we could say your role is at noon.

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Your salad is at 10 and 11.

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This is not a great picture to do a clock method on.

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Usually in the hospital, our plates are on a circle, so it's easy to do the clock, but we would use a clock position.

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We're not going to tell them to feel each item.

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Patients may not want to feel each item with their fingers.

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We don't say it's all on the plate and we don't just tell them the menu.

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We're going to use those clock positions.

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Which of our therapy involves no physical contact with the client?

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So massage and chiropractic care, this is going to use that actual touch.

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Therapeutic touch, even though it has the word touch, is our one that does not.

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Healing touch does have some touching.

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Guided imagery and meditation are examples of which type of therapy.

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So these would be our mind and body.

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Biological therapy, this is going to be more of our herbal supplements, our aromatherapies.

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Manual would be things like chiropractor, chiropractic care, acupuncture.

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Bioenergetic would be those healing touch, that therapeutic touch.

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So guided imagery and meditation are mind and body.

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Traditional Chinese medicine, I can't ever say this word so I'm going to try, are classified as these are alternative systems of care.

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So we just talked about these other three.

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Biologic would be herbal supplements, aromatherapies, manual would be our chiropractor, bioenergetic could be Reiki, healing touch, therapeutic touch.

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But these are going to be our alternative systems of care.

00:24:02

Which admission question best identifies A client's use of complementary therapies?

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Remember, we don't just want to say what medications do you take because they may not consider them a medication.

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We don't want to say you don't take any herbs, do you?

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That can sound judgmental.

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We don't want to ask about any risky alternative medicines because once again, that can sound like we're judging them.

00:24:22

We just want to say, do you take any vitamins, supplements, or herbs or herbal supplements?

00:24:27

You can also talk to them and say, here's what I have.

00:24:30

Is there anything else you take that might not be prescribed or anything that you buy over the counter at Walgreens or at a pharmacy or at a store?

00:24:38

We want to make sure we try to be very open and non-judgmental so that they'll open up and tell us what they're taking.

00:24:47

Gallbladder disease causing right shoulder pain is an example of which type of pain.

00:24:52

So sometimes when patients gallbladders hurt, they'll feel it in their belly, but they also feel it in their shoulder.

00:24:59

This is our referred pain.

00:25:01

I think there's a question next on one of these.

00:25:03

So I'm going to wait and we'll see.

00:25:05

If not, I'll switch back.

00:25:08

Yeah.

00:25:09

A client describes vague, poorly localized cramping deep in the abdomen.

00:25:13

So this is our visceral pain.

00:25:16

So referred pain we just talked about, somatic pain is going to be more like our joints.

00:25:21

our bones, our muscles.

00:25:24

Cutaneous pain is more that very light top of the skin, which is an appropriate comfort function goal.

00:25:35

So this is we talked about patient pain goals.

00:25:38

We know I want zero pain at all times.

00:25:40

Yes, we want you to have zero pain.

00:25:42

That is fine, but it may not be a realistic goal.

00:25:45

I don't want to take any medicine.

00:25:48

That's fine.

00:25:48

If you don't want to take in, you don't have to, but we might not meet your goal.

00:25:52

And that's not really a good comfort function goal.

00:25:55

Sleeping all day, also not a great goal.

00:25:57

But painted a three out of 10 so I can walk in the hall, that's a great goal.

00:26:01

If we can get their paint down to a three so they feel comfortable getting up, that's great.

00:26:09

A client is receiving IV morphine, which assessment is the priority.

00:26:13

So this would be like if they're getting in a PCA or they're getting it just through their IV.

00:26:19

Our biggest thing we're gonna watch for is respiratory rate and sedation level.

00:26:23

We are gonna watch their bowel sounds to make sure they're not getting constipated, but most important, respiratory rate and sedation level.

00:26:33

And then our last one, gabapentin can be added for a client's burning pain.

00:26:37

And this would be considered an adjunct analgesic.

00:26:40

So gabapentin, it can really be helped to treat different kinds of pain.

00:26:47

And it's just used as an adjunct.

00:26:48

It's not an opioid, it's not an NSAID, it's not necessarily something that everybody takes, but it can help with that neuropathic pain.

00:26:57

So it's considered an adjunct.

00:27:03

Okay, that is your Kahoot.

00:27:05

If you like this mode, you can do it on the learn mode.

00:27:08

The only problem is it doesn't shuffle them for you, but you can also do it solo and it should, it might shuffle them that way for you or there might be an option under this little cog wheel to shuffle them.

00:27:20

You can do flashcards with them and it'll have you the number and you can say you know or you don't know and you can flip it.

00:27:27

So you have some options for this.

00:27:30

Let me know if you have any questions.